Name	: Mr. SREE RAMI REDDY		
Lab No.	: 488193970	Age	: 44 Years
Ref By	: CHANDHAN KUMAR MD DM	Gender	: Male
Collected	: 10/2/2026 7:45:00AM	Reported	: 10/2/2026 5:24:23PM
A/c Status	: P	Report Status	: Final
Collected at	: SUN DIAGNOSTIC Kurnool Medicalcollege S.O, SUN DIAGNOST # 40-427, ABHILASH COMPLEX, NEAR ARUNODA	Processed at	: LPL Kurnool Shop no. 204, 2nd Floor, Padmavathi Venkateshwara Complex,Gayathri Estate, Near Vishwabharathi Hospital Kurnool-518002

Test Report

Test Name	Results	Units	Bio. Ref. Interval
HEMOGRAM (Spectrophotometry, Electrical Impedance, Analogical Integration, Calculated, Manual Microscopy & Westergren)			
Hemoglobin	**11.50**	g/dL	13.00 - 17.00
Packed Cell Volume (PCV)	**35.90**	%	40.00 - 50.00
RBC Count	**4.03**	mill/mm3	4.50 - 5.50
MCV	89.00	fL	83.00 - 101.00
Mentzer Index	22.1		
MCH	28.50	pg	27.00 - 32.00
MCHC	32.10	g/dL	31.50 - 34.50
Red Cell Distribution Width (RDW)	**25.40**	%	11.60 - 14.00
Total Leukocyte Count (TLC)	7.10	thou/mm3	4.00 - 10.00
Differential Leucocyte Count (DLC)			
Segmented Neutrophils	67.00	%	40.00 - 80.00
Lymphocytes	28.00	%	20.00 - 40.00
Monocytes	4.00	%	2.00 - 10.00
Eosinophils	1.00	%	1.00 - 6.00
Basophils	0.00	%	<2.00
Absolute Leucocyte Count			
Neutrophils	4.76	thou/mm3	2.00 - 7.00
Lymphocytes	1.99	thou/mm3	1.00 - 3.00
Monocytes	0.28	thou/mm3	0.20 - 1.00
Eosinophils	0.07	thou/mm3	0.02 - 0.50

If Test results are alarming or unexpected, client is advised to contact the Customer Care immediately for possible remedial action.
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Test Report

Test Name	Results	Units	Bio. Ref. Interval
Basophils	**0.00**	thou/mm3	0.02 - 0.10
Platelet Count	**141**	thou/mm3	150.00 - 410.00
Mean Platelet Volume	8.8	fL	6.5 - 12.0
E.S.R.	**34**	mm/hr	0 - 15

Comment

In anaemic conditions Mentzer index is used to differentiate Iron Deficiency Anaemia from Beta- Thalassemia trait. If Mentzer Index value is >13, there is probability of Iron Deficiency Anaemia. A value <13 indicates likelihood of Beta- Thalassemia trait and Hb HPLC is advised to rule out the Thalassemia trait.

Note

1. As per the recommendation of International council for Standardization in Hematology, the differential leucocyte counts are additionally being reported as absolute numbers of each cell in per unit volume of blood
2. Test conducted on EDTA whole blood

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Test Report

Test Name	Results	Units	Bio. Ref. Interval
LIVER FUNCTION TEST (LFT) (Spectrophotometry)			
AST (SGOT)	**114.2**	U/L	15.00 - 40.00
ALT (SGPT)	24.5	U/L	4.00 - 36.00
AST:ALT Ratio	**4.66**		<1.00
GGTP	**312.9**	U/L	9.00 - 54.00
Alkaline Phosphatase (ALP)	**276.20**	U/L	35.00 - 123.00
Bilirubin Total	1.16	mg/dL	0.10 - 1.20
Bilirubin Direct	**0.69**	mg/dL	<0.40
Bilirubin Indirect	0.47	mg/dL	<1.10
Total Protein	6.82	g/dL	6.40 - 8.30
Albumin	**3.31**	g/dL	3.50 - 5.20
Globulin(Calculated)	**3.51**	gm/dL	2.0 - 3.5
A : G Ratio	0.94		0.90 - 2.00

Note

1. In an asymptomatic patient, Non alcoholic fatty liver disease (NAFLD) is the most common cause of increased AST, ALT levels. NAFLD is considered as hepatic manifestation of metabolic syndrome.
2. In most type of liver disease, ALT activity is higher than that of AST; exception may be seen in Alcoholic Hepatitis, Hepatic Cirrhosis, and Liver neoplasia. In a patient with Chronic liver disease, AST:ALT ratio>1 is highly suggestive of advanced liver fibrosis.
3. In known cases of Chronic Liver disease due to Viral Hepatitis B & C, Alcoholic liver disease or NAFLD, Enhanced liver fibrosis (ELF) test may be used to evaluate liver fibrosis.
4. In a patient with Chronic Liver disease, AFP and Des-gamma carboxyprothrombin (DCP)/PIVKA II can be used to assess risk for development of Hepatocellular Carcinoma.
5. Test conducted in Serum

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Test Report

Test Name	Results	Units	Bio. Ref. Interval
KIDNEY FUNCTION TEST (KFT) (Spectrophotometry, Indirect ISE & Calculated)			
Creatinine	0.95	mg/dL	0.50 - 1.40
GFR Estimated	101	mL/min/1.73m2	>59
GFR Category	G1		
Urea	15.55	mg/dL	13.00 - 43.00
Urea Nitrogen Blood	7.26	mg/dL	6.00 - 20.00
BUN/Creatinine Ratio	8		
Uric Acid	6.71	mg/dL	3.50 - 7.20
Total Protein	6.82	g/dL	6.40 - 8.30
Albumin	**3.31**	g/dL	3.50 - 5.20
Globulin(Calculated)	**3.51**	gm/dL	2.0 - 3.5
A : G Ratio	0.94		0.90 - 2.00
Calcium, Total	8.85	mg/dL	8.50 - 10.40
Phosphorus	**2.32**	mg/dL	2.50 - 4.80
Sodium	142.20	mEq/L	136.00 - 145.00
Potassium	**3.15**	mEq/L	3.50 - 5.10
Chloride	97.10	mEq/L	97.00 - 108.00

Note

1. Estimated GFR (eGFR) calculated using the 2021 CKD-EPI creatinine equation and GFR Category reported as per KDIGO guideline 2012.
2. eGFR category G1 or G2 does not fulfil the criteria for CKD, in the absence of evidence of kidney damage
3. The BUN-to-creatinine ratio is used to differentiate prerenal and postrenal azotemia from renal

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Test Report

Test Name	Results	Units	Bio. Ref. Interval

azotemia. Because of considerable variability, it should be used only as a rough guide. Normally, the BUN/creatinine ratio is about 10:1

4. Test conducted in Serum

Dr.Pagidikalva Aparna
MD Pathology
Chief of Laboratory
Dr Lal PathLabs Ltd

----------End of report ----------

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IMPORTANT INSTRUCTIONS

•Test results released pertain to the specimen submitted.•All test results are dependent on the quality of the sample received by the Laboratory.•Laboratory investigations are only a tool to facilitate in arriving at a diagnosis and should be clinically correlated by the Referring Physician .•Report delivery may be delayed due to unforeseen circumstances. Inconvenience is regretted.•Certain tests may require further testing at additional cost for derivation of exact value. Kindly submit request within 72 hours post reporting.•Test results may show interlaboratory variations.•The Courts/Forum at Delhi shall have exclusive jurisdiction in all disputes/claims concerning the test(s) & or results of test(s).•Test results are not valid for medico legal purposes.•This is computer generated medical diagnostic report that has been validated by Authorized Medical Practitioner/Doctor.•The report does not need physical signature.

(#) Sample drawn from outside source.

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